Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

**Op. Dr. Murat Ayata Private Clinic**
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

INFORMED CONSENT FORM

# Ventilation Tube Insertion / Myringotomy

Insertion of grommets into the eardrum / opening the eardrum to drain fluid

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

### PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Side: ☐ Right ☐ Left ☐ Both

Planned procedure: ☐ Myringotomy + ventilation tube ☐ Myringotomy only (no tube)

Tube type: ☐ Short-term tube (grommet) ☐ Long-term tube (T-tube, etc.) _____

Additional procedures in the same session: ☐ Adenoidectomy ☐ Tonsillectomy ☐ None _____

## 1 Your condition and the aim of the procedure

The middle ear is a small space behind the eardrum that is normally filled with air. It is ventilated through the **Eustachian tube**, which opens into the back of the nose. When the Eustachian tube does not work well (especially in children, because of enlarged adenoids, allergy or frequent colds), **fluid collects** in the middle ear (“glue ear”).

This fluid can cause **reduced hearing**, and in children **delayed speech and language development**, attention problems, poor balance and **repeated ear infections**. If the fluid persists for a long time, the eardrum may become thin, retracted and permanently changed. Similar problems can occur in adults, for example after flights or diving, or because of conditions at the back of the nose.

The aim of this procedure is to make a small opening in the eardrum to drain the fluid and, with a small tube placed in it, to keep the middle ear ventilated — **improving hearing and reducing the frequency of infections.**

**Op. Dr. Murat Ayata Private Clinic**
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

## 2 How is the procedure performed?

In children the procedure is usually performed under a short general anaesthetic; in adults it is mostly done under local anaesthesia. It is carried out through the ear canal under a microscope; **there are no cuts or scars on the outside.**

1. **Cleaning the ear canal:** Under the microscope, any wax in the ear canal is removed and the eardrum is examined.
2. **Myringotomy:** A small cut of about 1–2 mm is made in a part of the eardrum where it will not affect hearing.
3. **Draining the fluid:** The fluid in the middle ear (which may be clear, sticky or infected) is removed with fine suction. If needed, a sample may be taken for culture.
4. **Placing the tube:** A tiny, spool-shaped tube a few millimetres in size is placed in the opening. This tube temporarily takes over the job of the Eustachian tube and keeps the middle ear ventilated.
5. **Finishing:** Ear drops are applied if needed. Usually no packing is placed in the ear, or only a small piece of cotton wool.

**Short-term tubes** are usually pushed out naturally within **6–18 months** as the eardrum heals, and fall into the ear canal; this often goes unnoticed. **Long-term tubes** stay in place for longer and sometimes need to be removed in a separate procedure.

When only a myringotomy is performed (without a tube), the opening in the eardrum closes within a few days and fluid may collect again.

| ANAESTHESIA | APPROXIMATE DURATION | HOSPITAL STAY / DISCHARGE |
|---|---|---|
| General anaesthesia in children; usually local anaesthesia in adults | About 10–20 minutes for both ears | Usually home the same day |

## 3 Expected benefits and likelihood of success

- Hearing usually improves immediately or within a short time
- In children, a positive effect on speech, language development, attention and school performance
- Fewer and milder ear infections; when an infection does occur, it can usually be treated with ear drops instead of antibiotics by mouth
- Lower risk of permanent damage to the eardrum; relief of pressure and fullness in the ear

As long as the tube is in place and open, hearing improves and ear infections become less frequent in the great majority of patients. If the Eustachian tube has matured sufficiently by the time the tube comes out, fluid usually does not return. However, in some children (about **20–30 in 100**) fluid collects again and **further tubes** may be needed.

## 4 Other treatment options

- **Watchful waiting:** Middle-ear fluid clears by itself within 3 months in many children. If the hearing loss is mild and there are no risk factors, monitoring with regular hearing tests is possible.
- **Autoinflation:** Especially in older children, blowing up a special balloon through the nose can help open the Eustachian tube.
- **Hearing aids:** For patients who do not want, or are not suitable for, surgery, these can be a temporary solution while the fluid persists.
- **Medicines:** Allergy or nose and sinus disease is treated if present; however, there is limited evidence that antibiotics, steroids or nasal sprays clear middle-ear fluid permanently.
- **Adenoidectomy:** In suitable children it may be recommended together with tubes or when further tubes are needed.

**Op. Dr. Murat Ayata Private Clinic**
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

## 5 What happens if the procedure is not performed?

Without the procedure, the fluid may clear by itself in some patients; if it persists, however, reduced hearing, delayed speech and language development in children, attention and school problems, poor balance and repeated ear infections may continue. Fluid that persists for a long time can lead to retraction of the eardrum, permanent eardrum changes, damage to the small hearing bones and, rarely, more serious ear disease requiring surgery, such as **cholesteatoma**.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

## 6 Preparing for the procedure

- A hearing test (audiometry) and a middle-ear pressure test (tympanometry) are carried out before the procedure.
- If a general anaesthetic is planned, you will be assessed by the anaesthetist; no solid food or milk should be given for **6 hours** before the operation, and for clear fluids follow the time given by the anaesthetist.
- Please let us know about any fever, cough or flu in the 2 weeks before the procedure; it may be postponed.
- If there is discharge from the ear or an active infection, it may need to be treated before the procedure.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

## 7 Recovery: what to expect

| | |
|---|---|
| **Day of the procedure** | Most patients go home within a few hours. There may be mild earache or a feeling of fullness, which settles with simple painkillers. A slightly blood-stained or watery discharge may come from the ear for a few days. Most patients **hear sounds louder straight away**; this can feel uncomfortable at first. |
| **First week** | Ear drops are used if your doctor has recommended them. Children can return to school or nursery, and adults to work, within 1–2 days. Take care to keep water out of the ears. |
| **Weeks 4–8** | The first follow-up examination is carried out; we check that the tube is in place and open, and the hearing test is repeated. |
| **While the tube is in (6–18 months)** | Check-ups are recommended about **every 3–6 months**. An ear infection shows up as discharge and is usually treated with ear drops. Flying is generally more comfortable while a tube is in place. |
| **After the tube comes out** | The eardrum is checked to make sure the hole has closed and there is no fluid in the middle ear; a hearing test is carried out. |

## 8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

**Op. Dr. Murat Ayata Private Clinic**
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

### Expected, temporary effects (not complications — a normal part of healing)

- Mild earache and a feeling of fullness (1–2 days)
- Slightly blood-stained or watery discharge from the ear for a few days
- Sounds temporarily seeming very loud or echoing
- Temporary nausea or restlessness after a general anaesthetic

### Common conditions (most are temporary or resolve with treatment)

- **Ear discharge (otorrhoea)** — About **10–25 in 100** patients with tubes have at least one episode of ear discharge, usually after a cold or water getting into the ear. It mostly clears within a few days with antibiotic ear drops. Rarely, the discharge may be persistent and require medicine by mouth, ear cleaning or removal of the tube.
- **White patches on the eardrum (tympanosclerosis)** — Chalk-like white patches on the eardrum are common (about **10–30+ in 100** ears). In most cases they do not affect hearing.
- **Early extrusion or blockage of the tube** — The tube may come out earlier than expected or become blocked with dried blood or discharge. Drops are used to try to clear it; if necessary, the tube is replaced.
- **Need for further tubes** — If fluid collects again after the tube comes out, about **20–30 in 100** children may need a second set of tubes (sometimes together with removal of the adenoids).

### Uncommon conditions

- **Permanent hole in the eardrum (perforation)** — After the tube comes out, the hole in the eardrum may not close: in about **1–3 in 100** patients with short-term tubes and more often with long-term tubes (about **10–20 in 100**). Small holes often cause no problems; if they lead to recurrent discharge or hearing loss, **closure by a separate operation (tympanoplasty/myringoplasty)** may be needed.
- **Thinning or retraction of the eardrum** — The eardrum may become thin or retract inwards where the tube was; follow-up may be needed.
- **Tube not coming out** — If the tube does not come out in the expected time (usually if it stays in longer than 2–3 years), it may need to be removed in a short procedure; in children this is usually done under general anaesthesia.
- **Tissue overgrowth in the ear canal (granulation)** — Red, inflamed tissue may form around the tube; it can cause blood-stained discharge and is treated with drops or a minor procedure.

### Rare or very rare, but potentially serious conditions

- **Tube moving into the middle ear** — Very rarely, instead of falling out, the tube may slip behind the eardrum into the middle ear; a separate procedure may be needed to remove it.
- **Cholesteatoma** — A cyst-like condition caused by the skin layer of the eardrum growing into the middle ear. It is **very rarely** associated with tube insertion. If it develops, surgical treatment is required.
- **Hearing loss, ringing in the ear (tinnitus) or dizziness** — Permanent hearing loss, tinnitus or dizziness due to involvement of the inner ear or hearing bones is **extremely rare**.
- **Injury to the ear canal and bleeding** — Small injuries can occur in narrow ear canals; very rarely, serious injuries related to anatomical variations of the facial nerve, inner ear or major blood vessels have been reported.

**Op. Dr. Murat Ayata Private Clinic**
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

**General risks that apply to any surgical / medical procedure**

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.
- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

## 9 When to contact us immediately

- Heavy or foul-smelling ear discharge lasting **more than 3–5 days** despite drops
- **Fever above 38.5 °C**, severe earache
- Redness or swelling behind the ear, or the ear being pushed forwards
- **Sudden hearing loss**, severe dizziness, unsteadiness or vomiting
- Facial asymmetry, drooping at the corner of the mouth or inability to close the eye
- Heavy bleeding from the ear

**If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).**

## 10 Medication and everyday-life advice

- While the tube is in place, **avoid dirty water (lakes, sea, swimming pools, bath water) getting into the ear.** Cotton wool with petroleum jelly or earplugs can be used in the shower and bath. Follow your doctor's advice about swimming; surface swimming with earplugs and/or a swimming cap is usually allowed, but diving is not recommended.
- Do not clean inside the ear canal with cotton buds.
- If ear discharge occurs, gently wipe it away from the outside, use the drops recommended for you and let your doctor know. Do not use other ear drops on your own; some drops are not suitable for the middle ear.
- Keep attending follow-up appointments even after the tube has come out; it is important to monitor hearing and the condition of the eardrum.

**Op. Dr. Murat Ayata Private Clinic**
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

## 11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

## 12 Photographs, image records and your personal data

**Photographs/videos** may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

☐ I agree that my images are used **only for medical records and treatment follow-up**.

☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).

☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.

☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

**Op. Dr. Murat Ayata Private Clinic**
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

## 13 Information and notes specific to you (completed by the doctor)

*Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write "none".*

## 14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.
- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that the tube will come out by itself after a certain time, that further tubes may be needed if fluid collects again, and that if a permanent hole remains in the eardrum, a separate operation may be needed to close it.

- ☐ **I GIVE MY CONSENT to the procedure described above.**
- ☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

*We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."*

**Op. Dr. Murat Ayata Private Clinic**
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

## 15 Signatures

| | FULL NAME | SIGNATURE | DATE / TIME |
|---|---|---|---|
| **Patient** | | | |
| **Legal representative (parent / guardian) — relationship:** | | | |
| **Doctor providing information and performing the procedure** | | | |
| **Interpreter (if needed) — language:** | | | |
| **Witness (if needed)** | | | |

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

### FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

### WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.